

SPEAK BEFORE THE REVEAL: PSYCHIATRY REFERENCE GUIDE

Post-session reference guide - dictate before ending the scribe recording

Before you end the recording, say your reasoning aloud so that *your* formulation, risk stance and plan (not the AI’s reconstruction) anchor the note. **Formulation, MSE, Risk and Plan** for all clinicians; **Key uncertainty, Differentials and Key phenomenology** additionally for trainees.

Formulation “My working formulation is...”

4 Ps Model	Predisposing - background vulnerabilities (genetics, early experience, temperament) Precipitating - current triggers; what brought them in now Perpetuating - maintaining factors (avoidance, relational patterns, environment) Protective - strengths, supports, resilience, coping resources
Psychodynamic Lens	Core conflict - e.g. need for closeness vs. fear of rejection Defence mechanisms - intellectualisation, humour, splitting, projection, avoidance Attachment style - secure / anxious-preoccupied / dismissive-avoidant / disorganised Repetition compulsion - re-enacting past relational patterns in therapy & life
CBT Snapshot	Thought–Feeling–Behaviour loop - identify the maintaining cycle Core beliefs - e.g. "I'm unlovable," "The world is unsafe" Cognitive distortions - catastrophising, black-and-white thinking, personalisation Behavioural experiments - planned tests of feared predictions

Preserves: Integrative synthesis: why this person, why now Guards against: A plausible pseudo-formulation reverse-engineered from surface content

Mental State Examination “Their mental state showed...”

Appearance & Behaviour	Grooming & self-care, dress, apparent age, physical signs · psychomotor activity (retardation / agitation) · abnormal movements (tremor, tardive dyskinesia, tics, stereotypies) · eye contact · rapport · attitude (cooperative / guarded / hostile / evasive) · level of distress.
Speech	Rate, rhythm, volume, tone, quantity, latency, spontaneity, articulation - e.g. pressured, slowed, poverty of speech, dysarthria, dysprosody.
Mood & Affect	Mood (stated): Euthymic, depressed, elevated, anxious, irritable, dysphoric, labile. Affect (observed): range, reactivity, congruence, quality (blunted / flat / restricted / full / expansive), appropriateness.
Thought Form & Content	Form: Linear & goal-directed, circumstantial, tangential, flight of ideas, loosening of associations, blocking, perseveration. Content: Preoccupations, obsessions, overvalued ideas, delusions (persecutory / grandiose / nihilistic / somatic), ideas of reference, ruminations, suicidal / homicidal ideation.
Perception & Cognition	Perception: hallucinations (auditory / visual / olfactory / tactile), illusions, depersonalisation, derealisation. Cognition: alertness, orientation, attention & concentration, short- and long-term memory, abstraction.
Insight & Judgement	Insight: awareness of illness, understanding of cause, attitude to treatment. Judgement: decision-making, awareness of consequences, risk awareness, capacity to keep self-safe.

Preserves: Integration of observed and conversational data Guards against: An MSE limited to only what was verbalized, since the AI cannot see the patient

Risk “The risk I’m holding in mind is...”

Levels adapted from Rao et al 2017 (chronic vs acute)[1]. *The crux is distinguishing acute change from chronic baseline: Applying acute / custodial responses to chronic risk is often iatrogenic.*

Low chronic	Stable, longstanding baseline risk at a low level. Managed through ongoing therapeutic support, not acute intervention.
High chronic	Persistently elevated baseline (recurrent self-harm, high-lethality history) with no recent change. Contained within the therapeutic frame; treating as acute (admission / custodial care) tends to be iatrogenic.
New / emerging	A risk not previously part of the picture. A change warranting fresh assessment and closer monitoring; may herald developing acute risk.
Acute (acute-on-chronic)	Escalation above the person’s baseline driven by acute precipitants (comorbid depression, substance use, recent discharge, acute stressor, perceived abandonment). The change that warrants active / protective response.

Assess: Chronic vs acute pattern (recent change) · Lethality · Chronic stressors · Current stressors · Protective factors.
Quality - Types of risk to consider: harm to self · harm to others · physical deterioration · mental-health deterioration · social / financial vulnerability.
Preserves: Dynamic risk formulation as a stance Guards against: Risk over-called from a metaphor, or under-called from non-obvious language

Plan “The agreed plan, and its reasoning, is...”

Consider: Action tied to the formulation (not a task list). **Psychotherapeutic:** therapeutic frame & relational stance · what is deliberately left open.
Pharmacological: medication rationale, including deliberate non-prescribing. **Assessment:** further examination/interview · investigations. **Next steps:** contingency / safety plan · follow-up interval and threshold to escalate · social support engagement
Preserves: Actions linked directly to the formulation Guards against: A superficial plan assembled only from spoken actions

Key uncertainty “I am least sure of...” trainees

Consider Competing formulations still open · missing collateral or information · what would change the plan · what to review next session.

Preserves: Calibrated doubt and a live differential **Guards against:** Declarative plans whose false confidence hides the gaps

Differential diagnosis “Differentials to examine include...” trainees

Consider Organic / medical mimics · substance-related · mood vs psychotic vs anxiety vs trauma vs personality · developmental (ASD / ADHD) in young people · what evidence would confirm or exclude each.
Preserves: Lateral diagnostic thinking **Guards against:** Generic, non-specific differentials

Key phenomenology “Key features supporting the formulation include...” trainees

Consider The specific symptoms / signs that anchor the leading hypothesis · their severity, duration and course · what remains a hypothesis versus what is established.
Preserves: Phenomenological reasoning and interpretive boundaries **Guards against:** Features of a hypothesis being recorded as an established diagnosis

Therapeutic Relationship Dynamics

Alliance & Engagement	Alliance strength - collaborative vs. resistant; degree of shared goals
	Ruptures & repairs - tension, withdrawal, misunderstanding; how addressed
	Boundary testing - lateness, cancellations, oversharing, prolonged silence
	Dependence vs. autonomy - reassurance-seeking vs. self-directed ownership
	Room affect - tension, warmth, humour, frustration, flatness, avoidance
Transference Patterns	<i>How the patient relates to you through past relational templates</i>
	Parental - critical father / nurturing mother / absent caregiver
	Peer / sibling - competition, camaraderie, rivalry, jealousy
	Authority figure - expecting judgment, punishment, or validation
	Rescuer / saviour - assumption you will fix everything; helplessness
Countertransference Awareness	Abandonment - hypervigilance to rejection, clinginess, pre-emptive withdrawal
	Idealisation / devaluation - oscillating between you as perfect vs. useless
	<i>Your emotional responses as clinical data</i>
	Protective / rescuing - urge to shield; difficulty holding boundaries
	Frustrated / irritated - feeling stuck, impatient, or provoked
	Over-invested - ruminating outside sessions; boundary blur
	Dismissive / numb - emotional disconnect, avoidance, boredom
	Anxious / self-doubting - fear of saying the wrong thing; performance worry
	Guilty / inadequate - sense of failing the patient; over-responsibility

Worked example

“**My working formulation is** that Sam’s presentation is a normative reaction to his parents’ separation, amplified by an anxious temperament and a school that has stopped scaffolding him - not a depressive disorder. **Their mental state showed** reactive affect, no psychomotor change, linear thought form, no perceptual disturbance. **The risk I’m holding in mind is** low chronic - no acute change, though I note new access to a sibling’s medication and am lowering my threshold to review. **The agreed plan, and its reasoning, is** to hold medication and begin family work first - the wait is the intervention - reviewing in two weeks, sooner if sleep worsens. **I am least sure** whether the irritability is depressive or an early bipolar profile; I’m watching, not committing.”

Blank prompt card Glance and dictate

Formulation: My working formulation is...
MSE: A+B · S · M+A · TF+TC · P+C · I+J
Risk: Level (low / high chronic · new-emerging · acute-on-chronic) + change from baseline · quality · lethality
Plan: The agreed plan, and its reasoning, is...
Key uncertainty (trainees): I am least sure of...
Differentials (trainees): Differentials to examine include...
Key phenomenology (trainees): Key features supporting the formulation include...

[1] S. Rao, J.H. Broadbear, K. Thompson, A. Correia, M. Preston, P. Katz, R. Trett, Evaluation of a novel risk assessment method for self-harm associated with Borderline Personality Disorder, *Australas Psychiatry* 25 (2017) 460–465. <https://doi.org/10.1177/1039856217707390>.
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